Your Name

Dr. Minard

AMNE 301

Translation Assignment #1: Tutorial 102

A 6-year-old boy was brought to the **surgery clinic for children** (pediatric) with a 2-year history of a stable lump on his neck **which caused no noticeable symptoms** (asymptomatic). Physical examination revealed a nontender, mobile, midline mass **at the front of the** (anterior) neck that moved up with **the act of sticking out his** (protrusion) tongue and swallowing. **Imaging using high-frequency sound waves** (ultrasonography) of the neck revealed a well-defined, thin-walled mass **that looked darker than the surroundings on the scan** (hypoechoic) and a normal **hormone-producing gland at the front of the neck** (thyroid gland). A **fluid-filled sac** (cyst) that **connects the tongue and developing thyroid gland** (thyroglossal duct) was suspected. A thyroglossal duct cyst is a common anomaly that **present at birth and located in the neck** (congenital cervical) that results from incomplete or failed **closure** (obliteration) of the thyroglossal duct **that present during early development before birth** (embryonic), which leaves behind fluid-secreting **layers of cells lining the duct** (epithelia) that generate a cyst. Imaging of the neck **using a strong magnetic field and radio waves** (Magnetic) (MRI) of the neck was subsequently conducted to plan the operative approach (although cross-sectional imaging is not typically needed for this condition in children). **Side-viewing** (sagittal) T2-weighted MRI showed a beaded duct **that appeared unusually bright on the scan** (hyperintense) and **formed an abnormal passage** (fistulous), originating from the tongue base, coursing **downwards** (inferiorly) through the **U-shaped** (hyoid) bone, and terminating in a cyst anterior to the thyroid cartilage. The cyst and the central portion of the hyoid bone were **excised** (surgically removed). Analysis of **the removed tissue under a microscope for signs of disease** (histopathological) showed no evidence of infection or cancer. The patient recovered well and had no recurrence at 2 years of follow-up.

From Donglai Hu and Junjie Chen, “Thyroglossal Duct Cyst,” New England Journal of Medicine 395:13 (30 Sept 2026), 1318, DOI: 10.1056/NEJMicm2607953.